Star Home Health Care Inc

OASIS

2415 E CAMELBACK RD STE 700706
PHOENIX AZ 85016
Phone: (480)931-6009 Fax:
NPI: 1326705476

STAFF INFO
Name: Lara, Toni RN
Patient INFO
Name: COLLINS, RICHARD
DOB: 1956-12-09

OASIS Assessment: OASIS E1 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number
037834

(M0014) Branch State
Branch State: **Arizona**

(M0016) Branch ID Number

(M0018) Attending Physician (NPI)
1659451946

Physician:
MILESKI, ROBERT

Address:
9941 N 95TH ST SUITE 101 SCOTTSDALE AZ 85258

☐ **Not Applicable**

(M0020) Patient ID number
146

(M0030) Start of Care Date
03/29/2026

(M0032) Resumption of Care Date
mm/dd/yyyy

☑ **Not Applicable**

(M0040) Patient Name

First Name:	Middle Name:	Last Name:	Suffix:
RICHARD		COLLINS	

(M0050) Patient State of Residence
State of Residence: **Arizona**

(M0060) Patient Zip Code
85255

(M0063) Medicare Number
5RK6RA0CW54

☐ **Not Applicable**

(M0065) Medicaid Number

☑ **Not Applicable**

(M0064) Social Security Number

☑ **Not Applicable**

(M0066) Birth Date *
12/09/1956

(M0069) Gender *
Gender: **Male**

(M0080) Discipline Completing Assessment:
● 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT

Assigned Staff:
Lara, Toni RN

(M0090) Date Assessment Completed:
03/29/2026

(M0100) Assessment Reason
● 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☑ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ Z. None of the above

(A1110) Language

A. What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
● 0 - No ○ 1 - Yes ○ 9 - Unable to determine

(M0102) Date of Physician-ordered Start of Care (Resumption of Care)
If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1250, Transportation, if date entered:
03/29/2026

☐ **NA - No specific SOC/ROC date ordered by physician**

(M0104) Date of Referral:
Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:
mm/dd/yyyy

(M0150) Current Payment Sources for Home Care: (Mark all that apply.) *

- [ ] 0. None; no charge for current services
- [x] 1. Medicare (Traditional fee-for-service)
- [ ] 2. Medicare (HMO/managed care/Advantage plan)
- [ ] 3. Medicaid (Traditional fee-for-service)
- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 6. Title programs (for example, Title III, V, or XX)
- [ ] 7. Other government (for example, TriCare, VA)
- [ ] 8. Private insurance
- [ ] 9. Private HMO/managed care
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1250) Transportation Has lack of transportation kept you from medical appointments, meetings, work, or from getting things needed for daily living?

- [ ] A. Yes, it has kept me from medical appointments or from getting my medications
- [ ] B. Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need
- [x] C. No
- [ ] X. Patient unable to respond
- [ ] Y. Patient declines to respond

(M1000) Inpatient Facilities that patient was discharged within the past 14 days? (Mark all that apply.)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [ ] 3. Short-stay acute hospital (IPPS)
- [ ] 4. Long-term care hospital (LTCH)
- [ ] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [x] 7. Other (specify): Honor health Hospital
- [ ] NA - Patient was not discharged from an inpatient facility _ **[Go to M1021]**

(M1005) Inpatient Discharge Date (most recent):

03/25/2026

- [ ] UK - Unknown

Primary Reason for Home Health/Medical Necessity:

This is a case of a 69-year-old male patient to Star home health care services following a total shoulder replacement surgery scheduled for March 24, 2026. His primary diagnosis is a left shoulder condition necessitating surgical intervention, post-surgery, the patient will require skilled nursing services for dressing changes and physical therapy to facilitate his recovery.

The patient is homebound due to his medical status post-surgery that include significant functional limitations, particularly in mobility and self-care activities due to pain, swelling, and the need for rehabilitation of the affected shoulder. His ability to perform daily activities will be compromised, necessitating support in the home environment. This aligns with Medicare's home health admission guidelines, which stipulate that patients must demonstrate a need for skilled care and have difficulty performing self-care tasks. The skilled care required includes physical therapy, which is essential for regaining strength and range of motion in the shoulder, and skilled nursing to manage post-operative care, including wound care and monitoring for any complications. The medical necessity of these services is underscored by the need for ongoing assessment and intervention to ensure proper healing and restoration of function following surgery. Furthermore, the patient's recent hospital discharge and the nature of his surgery highlight the need for a structured home health care plan. His recovery will benefit from the professional oversight of skilled nurses and therapists to mitigate risks, optimize rehabilitation efforts, and support his transition back to independence. Therefore, admitting this patient to home health care will provide the comprehensive support he requires to achieve optimal recovery outcomes in the comfort of his home.

Homebound Reason

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [ ] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [ ] Cane
  - [ ] Manual Wheel Chair
  - [ ] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [x] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

s/p total left shoulder replacement surgery

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadprilegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [ ] Unable to tolerate standing or walking for extended periods
- [x] Weakness and pain from recent surgery
- [ ] Experiencing pain impairs/restricts mobility
- [ ] Visually impaired-requires constant attendance/assist
- [ ] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [ ] Cognitively impaired, unable to safely leave home without assist
- [x] Leaving home is exhausting, extended rest periods to recover
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Extended respiratory recovery after exertion
- [ ] Narcotics impaired ability to safely leave home unassisted

- [ ] Requires human assist due to unsteady gait/poor balance
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [ ] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [x] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [ ] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [ ] Activities restricted due to pain limitations

Other Homebound Reason :

Prognosis

- ( ) 1-Poor
- ( ) 2-Guarded
- (x) 3-Fair
- ( ) 4-Good
- ( ) 5-Excellent

Functional Limitations

- ( ) 1 - Amputation
- ( ) 2 - Bowel/Bladder (Incontinence)
- ( ) 3 - Contracture
- (x) B- Other (Specify)
- ( ) 4 - Hearing
- ( ) 5 - Paralysis
- (x) 6 - Endurance
- (x) 7 - Ambulation
- ( ) 8 - Speech
- ( ) 9 - Legally blind
- ( ) A - Dyspnea with minimal exertion

Pain

Emergency Preparedness:

Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
Notified local utility companies and local emergency offices notified of life supporting equipment being used
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Discussed patient specific emergency planning options
Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Obtained the patient's emergency contact numbers for the medical record

Acuity Level:
Medium Risk or Medium Priority

Emergency Triage:
Category 4 - Visit could be postponed 72-96 hours without adverse effects

Additional Emergency Preparedness Information

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [ ] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives

Patient has Advance Directives? ( ) Yes (x) No

If you have checked "yes", fill the following fields:

- [ ] Healthcare Representative
- [ ] Do Not Resuscitate
- [ ] Power of Attorney
- [ ] Living Will
- [ ] The patient has not executed any advance directives to health care.
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [x] Full Code — Copies to Follow
- [ ] The patient was provided with written and verbal information on advance directives.

Comments:

Has surrogate? ( ) Yes (x) No

Surrogate Name:

Surrogate Phone:

Cultural

Primary Language? [x] English [ ] Spanish [ ] Chinese [ ] Vietnamese

Other:

Is religion important to the patient? ( ) Yes (x) No

Patient's religious preference:

Patient's primary source of emotional support:

Past Medical History (Mark all that apply)

- [ ] CHF
- [ ] Chest Pain
- [x] HTN
- [ ] Cancer:
- [ ] Osteoathritis/DJD:
- [ ] Cardiomyopathy
- [ ] MI
- [ ] PVD
- (Specify Type):
- (Specify site affected):
- [ ] Arrhythmia
- [ ] CAD
- [ ] Murmur
- In remission? ( ) Yes ( ) No

- [x] Joint Replacement: (Specify joint): left shoulder

- [ ] Rheumatoid Arthritis
- [ ] Gait Problems
- [ ] Fractures
- [x] Falls
- [ ] CVA
- [x] TIA
- [ ] MS
- [ ] Hemiplegia
- [ ] Seizures
- [ ] Headaches
- [ ] Dizziness/Vertigo

- [ ] IBS
- [ ] Crohn's Disease
- [ ] Diverticulitis/Diverticulosis
- [ ] Constipation
- [ ] Diarrhea
- [ ] Fecal Incontinence
- [ ] Liver/Gallbladder Problems: (Specify):

- [ ] Depression
- [ ] Anxiety
- [ ] Dementia
- [ ] Substance Abuse: (Specify):
- [ ] Alzheimer's
- [ ] Mental Disorder: (Specify):

- [ ] Pressure Ulcer
- [ ] Stasis Ulcer
- [ ] Diabetic Ulcer
- [ ] Trauma Wound
- [x] Other: (Specify): Surgical wound at left shoulder, Hyperlipidemia,
- [ ] Chronic Kidney Disease
- [ ] Renal Failure
- [ ] Dialysis

- [ ] Anemia
- [ ] Abnormal Coagulation
- [ ] Blood Clots
- [ ] Diabetes
- [ ] Thyroid Problems

- [ ] COPD
- [ ] Asthma
- [ ] Emphysema
- [ ] Chronic Obstructive Bronchitis
- [ ] Chronic Obstructive Asthma

- [ ] Urinary Incontinence
- [ ] Urinary Retention
- [x] BPH
- [ ] Recent/Frequent UTI
- [ ] Tuberculosis
- [ ] Hepatitis: (Specify):
- [ ] Infectious Disease: (Specify):

- [ ] Tobacco Dependence

Type: | Amount: | Length of Time Used:

Past Surgical History:
Presence of left artificial shoulder joint

Health Screening

Last Cholesterol Level: mm/dd/yyyy

Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ◯ Yes ◯ No

Last Pap Smear: mm/dd/yyyy

Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy

Last Colonoscopy: mm/dd/yyyy

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	◯ Patient Lives alone around the clock	◯ Patient Lives alone Regular daytime	◯ Patient Lives alone Regular Night time	◯ Patient Lives alone Short-term Assistance	◯ Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	● Patient lives with other person(s) in the home Around the clock	◯ Patient lives with other person(s) in the home Regular daytime	◯ Patient lives with other person(s) in the home Regular Night time	◯ Patient lives with other person(s) in the home Short-term Assistance	◯ Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	◯ Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	◯ Regular daytime	◯ Patient lives in congregate situation Regular Night time	◯ Patient lives in congregate situation Short-term Assistance	◯ Patient lives in congregate situation No Assistance available

Caregiver Contact Info:

Primary Caregiver:	Relationship to Patient	Contact Number
Elizabeth	Daughter	602-402-9978

Comments:

Name of Facility:

Phone:

Able to safely care for patient? ● Yes ◯ No

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

OASIS IS ALREADY EXPORTED, PRIMARY DIAGNOSIS AND OTHER DIAGNOSIS FROM 1 TO 5 ARE NOW DISABLED

	Onset Date	Severity	
PRIMARY DIAGNOSIS Z47.1 - Aftercare following joint replacement surgery	03/29/2026	○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^	○ Onset ◉ Exacerbate
OTHER DIAGNOSIS - 1 Z96.612 - Presence of left artificial shoulder joint	03/29/2026	○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^	○ Onset ◉ Exacerbate
OTHER DIAGNOSIS - 2 I10. - Essential (primary) hypertension	03/29/2026	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^	○ Onset ◉ Exacerbate
OTHER DIAGNOSIS - 3 E78.2 - Mixed hyperlipidemia	03/29/2026	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^	○ Onset ◉ Exacerbate
OTHER DIAGNOSIS - 4 N40.0 - Benign prostatic hyperplasia without lower urinry tract symp	03/29/2026	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^	○ Onset ◉ Exacerbate
OTHER DIAGNOSIS - 5 Z86.73 - Prsnl hx of TIA (TIA), and cereb infrc w/o resid deficits	03/29/2026	○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^	◉ Onset ○ Exacerbate
Z91.81 - History of falling	03/29/2026	○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^	◉ Onset ○ Exacerbate

(M1028) Active Diagnoses – Comorbidities and Co-existing Conditions – Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.

- [ ] **1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)**
- [ ] **2. Diabetes Mellitus (DM)**
- [x] **3. None of the above**

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up

A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
71

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
265

BMI:
36.96

Comments:

Vital Signs

Temperature: 98.1 Temporal

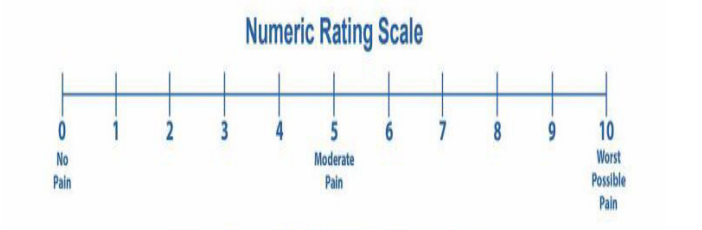
Pulse (per minute): Apical = Radial = 73

Respiratory (per minute): 18
Blood Pressure mm/Hg (R): 129 / 78 sitting
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 97
Comments:

Weight: 265 reported
Blood Pressure mm/Hg (L): /

O2 Sat in: Room Air

Pain Assessment



4

Collected Using: ☐ Face Scale ☑ Numeric Rating Scale

Pain Location and Description:

1 **Location**
Left schoulder

Start Date
03/24/2026

Activity
With Activity×

What makes pain worse?
Movement×

Intensity
4×

Duration

What makes pain better?
Med/Rest×

Does pain affect quality of life?

Unable to Assess ☐

Description
Throbbing×

Frequency
Frequent×

Unable to Assess ☐

Type of Intervention
Opiod Analgesic×

How well it is working
Very Well×

Status
New×

Comments:

(J0510) Pain Effect on Sleep

Ask patient: "Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"

- ○ **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ○ **1. Rarely or not at all**
- ● **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0520) Pain Interference with Therapy Activities

Ask patient: "Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"

- ○ **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ● **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0530) Pain Interference with Day-to-Day Activities

Ask patient: "Over the past 5 days, how often you have limited your day-to-day activities (excluding rehabilitation therapy session) because of pain?"

- ● **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost Constantly**
- ○ **8. Unable to answer**

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking	2. Indication Noted
	1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	■
E. Anticoagulant	☐	■
F. Antibiotic	☐	■
H. Opioid	☑	☑
I. Antiplatelet	☑	☑
J. Hypoglycemic (including insulin)	☐	■
Z. None of the Above	☐	

Medication

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	03/26/2026	oxyCODONE (Oral Pill)	10 mg 1 Tab Every four hours	By mouth	OPIOID ANALGESICS / Pain	New

	Start Date		Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	03/26/2026	CRESTOR	20 mg 1 Tab At bedtime	By mouth	ANTILIPEMIC AGENTS / Cholesterol	New
☐	03/26/2026	Tamsulosin (Oral Pill)	0.4 mg 2 caps Once a day	By mouth	ALPHA BLOCKERS/RELATED /	New
☐	03/26/2026	Acetaminophen (Oral Pill)	325 mg 2 Tab Every four hours	By mouth	NON-OPIOID ANALGESICS / pain	New
☐	03/26/2026	Aspirin (Oral Pill)	81 mg 1 Tab Once a day	By mouth	NON-OPIOID ANALGESICS / preventative	New
☐	03/26/2026	Clopidogrel (Oral Pill)	75 mg 1 Tab Once a day	By mouth	PLATELET AGGREGATION INHIBITORS /	New
☐	03/26/2026	Cyclobenzaprine (Oral Pill)	10 mg 1 Tab Three times a day	By mouth	SKELETAL MUSCLE RELAXANTS / muscle spasms	New

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

IV

Does patient have IV access? ◯ **Yes** ● **No**

(M2001) Drug Regimen Review

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **0. No – No issues found during review [Go to M2010, Patient/Caregiver High-Risk Drug Education]**
- ◯ **1. Yes – Issues found during review**
- ◯ **Not assessed/no information**
- ◯ **9. NA – Patient is not taking any medications [Go to O0110, Special Treatments, Procedures, and Programs]**

(M2003) Medication Follow-up

Did the agency contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- ◯ **0. No**
- ◯ **1. Yes**
- ◯ **Not assessed/no information**

(M2010) Patient/Caregiver High-Risk Drug Education

Has the patient/caregiver received instruction on special precautions for all high-risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ◯ **0. No**
- ● **1. Yes**
- ◯ **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

(M2020) Management of Oral Medications

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. (NOTE: This refers to ability, not compliance or willingness.)

- ◯ **0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ● **1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ◯ **2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ◯ **3. Unable to take medication unless administered by another person**
- ◯ **NA - No oral medications prescribed**

(M2030) Management of Injectable Medications

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ● **0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ◯ **1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ◯ **2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ◯ **3. Unable to take injectable medication unless administered by another person**
- ◯ **NA - No injectable medications prescribed.**

(O0110) Special Treatments, Procedures, and Programs

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10. Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐
C2. Continuous	☐
C3. Intermittent	☐

C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations

Pneumonia
◯ Yes ◯ No ● Unknown **Date:** mm/dd/yyyy

Flu
◯ Yes ◯ No ● Unknown **Date:** mm/dd/yyyy

Tetanus
◯ Yes ◯ No ● Unknown **Date:** mm/dd/yyyy

TB
◯ Yes ◯ No ● Unknown **Date:** mm/dd/yyyy

TB Exposure
◯ Yes ◯ No ● Unknown **Date:** mm/dd/yyyy

Hepatitis B
◯ Yes ◯ No ● Unknown **Date:** mm/dd/yyyy

Additional Immunizations

Immunization Name:
◯ Yes ◯ No ◯ Unknown **Date:** mm/dd/yyyy

Immunization Name:
◯ Yes ◯ No ◯ Unknown **Date:** mm/dd/yyyy

Comments:
Covid 19 vaccination unknown. Patient cannot recall exact date of last immunizations. Taught patient/PCG on the importance of up-to-date immunizations.

Vaccination Record

1st Dose COVID-19
Product Name/Manufacturer (Lot No.): **Date:** mm/dd/yyyy **Healthcare Professional or Clinic Site:**

2nd Dose COVID-19
Product Name/Manufacturer (Lot No.): **Date:** **Healthcare Professional or Clinic Site:**

mm/dd/yyyy

Booster 1
Product Name/Manufacturer (Lot No.): | **Date:** mm/dd/yyyy | **Healthcare Professional or Clinic Site:**

Booster 2
Product Name/Manufacturer (Lot No.): | **Date:** mm/dd/yyyy | **Healthcare Professional or Clinic Site:**

Allergies

Add Allergies

Name	Reaction	Start Date
Meperidine	Nausea	03/27/2026
Penicillians	Severe family hx of PCN allergy-patient has	03/27/2026

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Conditions of Participation
Neuromascular
Impairment:

- [x] No Problems identified
- [ ] Speech
- [ ] Hearing
- [ ] Visual
- [ ] Tremors
- [ ] Numbness
- [ ] Tingling
- [ ] Vertigo
- [ ] Ataxia
- [ ] Decreased Sensitivity

Others (Specify):

Psychosocial

- [x] WNL (Within Normal Limits)
- [ ] Poor Home Environment
- [ ] Impaired Decision Making
- [ ] Poor Coping Skills
- [ ] Demonstrated/Expressed Anxiety
- [ ] Agitated
- [ ] Inappropriate Behavior
- [ ] Depressed Mood
- [ ] Irritability
- [ ] Document as Clinical Narrative

Educational level: **College**

Primary Language: **English**

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language
- ○ Needs interpreter

Learning
- ○ Mental
- ○ Physical
- ○ Psychosocial
- ○ Functional

Emotions Reported:
- ○ Angry
- ○ Discouraged
- ○ Withdrawn
- ○ Flat affect
- ○ Difficulty coping
- ○ Disorganized
- ○ Depressed
- ○ Recent
- ○ Long Term

Treatment:

Evidence of abuse/neglect/exploitation:
- ○ Potential
- ○ Verbal/Emotional
- ○ Physical
- ○ Actual
- ○ Financial
- ● No Evidence of Abuse/Neglect/Exploitation

Unable to cope with altered health status, patient:
- ○ Lacks motivation
- ○ Has unrealistic expectations
- ○ Denies problems
- ○ Unable to recognize problems

Sleep/Rest
- ● Adequate
- ○ Inadequate

No. of hours slept per night: 7

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician ☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☑ Yes ☐ No ☐ NA

Did the patient have a job before admission? ○ Yes ◉ No

If yes, do they want to drive post-discharge? ☑ Yes ☐ No

If yes, do they want to return to work post-discharge? ○ Yes ○ No

(B1300) Health Literacy

How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

- ◉ **0. Never**
- ○ **1. Rarely**
- ○ **2. Sometimes**
- ○ **3. Often**
- ○ **4. Always**
- ○ **7. Patient declines to respond**
- ○ **8. Patient unable to respond**

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ○ **No (patient is rarely/never understood)** ◉ **Yes** ○ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◉ **None** ○ **One** ○ **Two** ○ **Three** ○ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year

- ○ **0. Missed by > 5 years or no answer**
- ○ **1. Missed by 2-5 years**
- ○ **2. Missed by 1 year**
- ◉ **3. Correct**
- ○ **Not assessed/no information**

Ask patient: **"What month are we in right now?"**

B) Able to report correct month

- ○ **0. Missed by > 1 month or no answer**
- ○ **1. Missed by 6 days to 1 month**
- ◉ **2. Accurate within 5 days**
- ○ **Not assessed/no information**

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week

- ○ **0. Incorrect or no answer**
- ◉ **1. Correct**
- ○ **Not assessed/no information**

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"

- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("something to wear")**
- ◉ **2. Yes, no cue required**
- ○ **Not assessed/no information**

B) Able to recall "blue"

- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("a color")**
- ◉ **2. Yes, no cue required**
- ○ **Not assessed/no information**

C) Able to recall "bed"

- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("a piece of furniture")**
- ◉ **2. Yes, no cue required**
- ○ **Not assessed/no information**

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** **12**

Mental Status

- ◉ **1 - Oriented**
- ○ **2 - Comatose**
- ○ **3 - Forgetful**
- ○ **4 - Depressed**
- ○ **5 - Disoriented**
- ○ **6 - Lethargic**
- ○ **7 - Agitated**
- ◉ **8 - Alert**
- ○ **9 - Confused**
- ○ **10 - Dementia**
- ○ **11 - Assaultive**
- ○ **12 - Delirious**
- ○ **13 - Wanders**
- 14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient is alert and oriented ×4. Reports feeling well this shift and expresses motivation to continue with the rehabilitation program. Denies pain, dizziness, or new concerns. States understanding of current therapy goals and plan of care. Receptive to education and engages appropriately in conversation.

Has there been a sudden/acute change in their mental status? ☐ Yes ☑ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☐ No ☐ Yes

Comments:

(C1310) Signs and Symptoms of Delirium (from CAM©)

A. Acute Onset of Mental Status Change

Is there evidence of an acute change in mental status from the patient's baseline?

◉ **0. No** ○ **1. Yes**

B. Inattention

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- ◉ **0. Behavior not present**
- ○ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

C. Disorganized thinking

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- ◉ **0. Behavior not present**
- ○ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

D. Altered level of consciousness

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- **vigilant** – startled easily to any sound or touch □ lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch
- **stuporous** – very difficult to arouse and keep aroused for the interview
- **comatose** – could not be arouse

- ◉ **0. Behavior not present**
- ○ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

(D0150) Patient Mood Interview (PHQ-2 to 9)

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**

If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**

Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence

- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency

- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	No ×	Never or 1 day
B. Feeling down, depressed, or hopeless	No ×	Never or 1 day
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		0

(D0700) Social Isolation

How often do you feel lonely or isolated from those around you?

- [x] 0. Never
- [ ] 1. Rarely
- [ ] 2. Sometimes
- [ ] 3. Often
- [ ] 4. Always
- [ ] 7. Patient declines to respond
- [ ] 8. Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed): (Mark all that apply.

- [ ] 1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- [ ] 2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- [ ] 3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- [ ] 4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- [ ] 5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
- [ ] 6. Delusional, hallucinatory, or paranoid behavior
- [x] 7. None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- [x] 0. Never
- [ ] 1. Less than once a month
- [ ] 2. Once a month
- [ ] 3. Several times each month
- [ ] 4. Several times a week
- [ ] 5. At least daily

(M1700) Cognitive Functioning

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- [x] 0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.
- [ ] 1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.
- [ ] 2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility
- [ ] 3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- [ ] 4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- [x] 0. Never
- [ ] 1. In new or complex situations only
- [ ] 2. On awakening or at night only
- [ ] 3. During the day and evening, but not constantly
- [ ] 4. Constantly
- [ ] 5. Patient nonresponsive

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- [x] 0. None of the time
- [ ] 1. Less often than daily
- [ ] 2. Daily, but not constantly
- [ ] 3. All of the time
- [ ] NA - Patient nonresponsive

Eyes and Ears

(B1000) Vision

Ability to see in adequate light (with glasses or other visual appliances)

- [x] 0. Adequate – sees fine detail, such as regular print in newspapers/books
- [ ] 1. Impaired – sees large print, but not regular print in newspapers/books
- [ ] 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- [ ] 3. Highly impaired – object identification in question, but eyes appear to follow objects
- [ ] 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision cont.

- [x] No Problems Identified
- [ ] Glasses
- [ ] Macular Degeneration
- [ ] Blurred Vision
- [ ] Glaucoma
- [ ] Contact Lenses:
- [ ] Right
- [ ] Left
- [ ] Cataracts
- [ ] Blindness(Specify):
- [ ] Others (Specify):

Comments:

Pupils

- [x] No Problems Identified
- [ ] PERRLA
- [ ] Anisocoria
- [ ] Right
- [ ] Left
- [ ] Others(Specify):

Comments:

(B0200) Hearing

Ability to hear (with hearing aid or hearing appliances if normally used)

- [x] 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- [ ] 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- [ ] 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- [ ] 3. Highly impaired – absence of useful hearing

Hearing cont.

- [x] No Problems Identified
- [ ] Right Hearing Impairment
- [ ] Left Hearing Impairment
- [ ] Deaf
- [ ] Speech Impairment (Specify):

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Heart Sounds

- [x] Normal
- [ ] Regular
- [ ] Irregular
- [ ] Murmur
- [ ] Gallops
- [ ] Clicks
- [ ] Friction Rule

Comments (If Abnormal (Explain)):

Edema

Upper:
- [ ] Right Extremities
- [ ] Left Extremities

Lower:
- [ ] Right Extremities
- [ ] Left Extremities

Others:

Comments:

.
- [ ] Cramps
- [ ] Claudication
- [ ] Fatigue

Capillary Refill:
- [x] > 3 secs
- [ ] < 3 secs

PT/INR: /

Peripheral Pulses

Radial:	[x] Strong/Bounding	[ ] Weak	[ ] Thready	[ ] Absent
Pedal:	[x] Strong/Bounding	[ ] Weak	[ ] Thready	[ ] Absent

Comments:

Chest/Thorax

Chest Pain

- [x] Denies
- [ ] Anginal
- [ ] Postural
- [ ] Localized
- [ ] Substernal
- [ ] Sharp
- [ ] Dull
- [ ] Aching
- [ ] Radiating
- [ ] Viselike
- [ ] Stabbing
- [ ] Others (Specify):

Associated with:
- [ ] Shortness of Breath
- [ ] Activity
- [ ] Rest
- [ ] Frequency/Duration:

Comments:

(M1400) When is the patient dyspneic or noticeably Short of Breath?

- [x] 0. Patient is not short of breath
- [ ] 1. When walking more than 20 feet, climbing stairs
- [ ] 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- [ ] 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- [ ] 4. At rest (during day or night)

Respiratory Status

- [ ] Dyspnea:
- [ ] At Rest
- [ ] With Exertion/Activity
- [ ] Accessory Muscle Used
- [ ] Orthopnea
- [ ] Retraction
- [ ] Stridor

O2 lpm O2 Saturation %
- [ ] PRN
- [ ] Continuous

- [ ] Mechanical Ventilator:
- [ ] Intubated
- [ ] Tracheostomy

Others (Specify):

Comments:

Lung Sounds

Anterior: Right: **Clear** Left: **Clear**

Posterior: Upper Right: **Clear** Upper Left: **Clear** Lower Right: **Clear** Lower Left: **Clear**

Comments:

Cough

- [x] None
- [ ] Dry
- [ ] Acute
- [ ] Chronic
- [ ] Non-Productive
- [ ] Productive:
- [ ] Not Assessed/No Information
- [ ] Yes
- [ ] No

Suction:
- [ ] Not Assessed/No Information
- [ ] Yes
- [ ] No

- [ ] Hemoptysis

Frequency: Amount:

Comments:

GASTROINTESTINAL & GENITOURINARY ASSESSMENT

Gastrointestinal (Mouth - Anus)

Mouth and Throat

- [x] No Problems Identified
- [ ] Dentures:
- [ ] Upper
- [ ] Lower
- [ ] Partial
- [ ] Bleeding Gums/Gingivitis
- [ ] Toothache
- [ ] Ulcerations
- [ ] Lesion

☐ Dysphagia ☐ Hoarseness ☐ Sore Throat ☐ Masses/Tumors

Comments:

Appetite

☐ Good ☑ Fair ☐ Poor Intake: ☑ Adequate ☐ Inadequate

Abnormalities:

☐ Anoxeria ☐ Cachexia ☐ Nausea ☐ Vomiting

☑ Decreased Appetite ☐ Reflux/Indigestion ☐ Diarrhea ☐ Constipation

☐ Bowel Incontinence ☐ Other (Specify):

Comments:
Patient is eating, though intake is slightly reduced compared to baseline. Decreased appetite appears related to recent surgery. Patient denies nausea or vomiting and is tolerating meals without difficulty. Hydration status adequate. Encouraged small, frequent meals and monitoring of intake.

(M1870) Feeding or Eating

Current ability to feed self meals and snacks safely. Note: This refers only to the process of ***eating***, ***chewing***, and ***swallowing***, not preparing the food to be eaten.

- ● **0. Able to independently feed self.**
- ○ **1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.**
- ○ **2. Unable to feed self and must be assisted or supervised throughout the meal/snack.**
- ○ **3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.**
- ○ **4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.**
- ○ **5. Unable to take in nutrients orally or by tube feeding.**

Feeding Cont.

☐ Formula ☐ Continuous Feeding cc: hours:

☐ Bolus: ☐ Gravity ☐ Pump cc: every hour(s):

Flushing Protocol:

PEG/G-Tube Placement:

☐ Placement Checked ☐ Residual Checked **Amount:**

Comments:

(K0520) Nutritional Approaches

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	☐
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	☐
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	☐
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	☐
Z. None of the above	☑

#16 Nutritional Requirements

☐ NPO ☐ No Added Salt ☐ Low Salt ☐ Low Fat ☐ Low Cholesterol ☐ No Concentrated Sweets

☐ Controlled Carbohydrates ☑ Regular ☐ Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
○ Without reason, has lost more than 10 lbs in the last 3 months	15	○ Does not always have enough money to buy foods need	10
○ Has an illness or condition that made pt change the type and/or amount of food eaten	10	○ Eats alone most of the time	5
○ Has open decubitus, ulcer, burn or wound	10		
○ Eats fewer than 2 meals a day	10	● Takes 3 or more prescribed or OTC medications a day	5
○ Has a tooth/mouth problem that makes it hard to eat	10	○ Is not always physically able to cook and/or feed self and has no caregiver to assist	5
○ Has 3 or more drinks of beer, liquor or wine almost every day	10	○ Frequently has a diarrhea or constipation	5
		Total	5

● Good Nutritional Status (Score 0 - 25) ○ Moderate Nutritional Status (Score 25 - 54) ○ High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:
Patient is eating, though intake is slightly reduced compared to baseline. Decreased appetite appears related to recent surgery. Patient denies nausea or vomiting and is tolerating meals without difficulty. Hydration status adequate. Encouraged small, frequent meals and checking of intake.

Weight

☑ No Change ☐ Weight Gain ☐ Weight Loss

Abdomen

☑ Abdominal Soft/Tender ☐ Abdominal Distention ☐ Girth(cm):

Comments:

(M1620) Bowel Incontinence Frequency

- [x] 0. Very rarely or never has bowel incontinence
- [ ] 1. Less than once weekly
- [ ] 2. One to three times weekly
- [ ] 3. Four to six times weekly
- [ ] 4. On a daily basis
- [ ] 5. More often than once daily
- [ ] NA - Patient has ostomy for bowel elimination
- [ ] UK - Unknown

(M1630) Ostomy for Bowel Elimination

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

- [x] 0. Patient does not have an ostomy for bowel elimination
- [ ] 1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.
- [ ] 2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

Ostomy/Stoma Appearance

- [ ] Pinkished-Red
- [ ] Moist
- Abnormalities:
- [ ] Purple
- [ ] Black in Color
- [ ] Dry
- [ ] Gray
- [ ] Bad Odor
- [ ] Itches/Hurt
- Other (Specify):

Bowel Sounds:

Abdominal Quadrants:

Right Lower	Right Upper	Left Lower	Left Upper	Last Bowel Movement:
Normoactive	Normoactive	Normoactive	Normoactive	03/27/2026

Comments:

Constipation

- [ ] Chronic
- [x] Acute
- [ ] Occasional
- [ ] Internal Hemorrhoids
- [ ] External Hemorrhoids
- [ ] Laxative/Enema Use

Comments:
Patient is currently taking prescribed opioids for post-operative pain management. Due to the increased risk of opioid-related constipation, patient is on a scheduled bowel regimen and is compliant with medications as ordered. Patient reports staying well-hydrated and is encouraged to continue adequate fluid intake to support bowel function. No reports of abdominal discomfort, nausea, or difficulty with bowel movements at this time.

Stool Appearance

- [x] Within Normal Limits
- [ ] Gray
- [ ] Tarry
- [ ] Black
- [ ] Fresh Blood
- [ ] Others (Specify):

Comments:

Genitourinary (Genitals & Urinary)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

- [x] 0. No
- [ ] 1. Yes
- [ ] NA - Patient on prophylactic treatment
- [ ] UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

- [x] Patient is incontinent
  - [ ] Day time
  - [ ] Night time
  - [ ] Day and Night
  - [ ] Timed-voiding defers incontinence
  - [ ] Occasional stress incontinence
- [ ] No incontinence or catheter (includes anuria or ostomy for urinary drainage)
- [ ] Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)

Genitourinary Assessment

- [x] Within Normal Limits

Abnormalities:

- [ ] Frequency
- [ ] Dysuria
- [ ] Urgency
- [ ] Anuria
- [ ] Nocturia
- [ ] Oliguria
- [ ] Retention
- [ ] Hematuria
- [ ] Irrigation
- [ ] Incontinence
- [ ] Diapers Used
- [ ] Penile Discharge
- [ ] Vaginal Bleeding/Discharge
- Others (Specify):

Urine Color:

- [ ] Clear
- [ ] Transparent
- [x] Pale Yellow
- [ ] Brownish Orange
- [ ] Pinkish Red
- [ ] Dark Yellow
- [ ] Blue or Green
- [ ] Foamy
- [ ] Transparent Yellow

Urine Consistency:

- [ ] Hematuria
- [ ] Sediment
- [ ] Odorous
- [ ] Cloudy

Urinary Catheter:

- [ ] External Catheter
- [ ] Foley
- [ ] Suprapubic
- [ ] Nephrostomy

Last Catheter Change: mm/dd/yyyy

Fr:

cc:

Comments:

INTEGUMENTARY (SKIN & WOUND)

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- [x] 0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**
- [ ] 1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non-removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^

F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury ^

(M1322) Current Number of Stage 1 Pressure Injuries:
Intact skin with non-blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persistent blue or purple hues.
Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:
(Excludes pressure ulcer/injury that cannot be staged due to a non-removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)
Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?
- [x] 0. No _[GO TO M1340]
- [ ] 1. Yes, patient has BOTH observable and unobservable stasis ulcers
- [ ] 2. Yes, patient has observable stasis ulcers ONLY
- [ ] 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non-removable dressing/device) _[GO TO M1340]

(M1332) Current Number of Stasis Ulcer(s) that are Observable:
- [ ] 1. One
- [ ] 2. Two
- [ ] 3. Three
- [ ] 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable
- [ ] 1. Fully granulating
- [ ] 2. Early/partial granulation
- [ ] 3. Not healing

(M1340) Does this patient have a Surgical Wound?
- [ ] 0. No _[GO TO N0415]
- [ ] 1. Yes, patient has at least one observable surgical wound
- [x] 2. Surgical wound known but not observable due to non-removable dressing/device _[GO TO N0415]

(M1342) Status of Most Problematic Surgical Wound that is Observable
- [ ] 0. Newly epithelialized
- [ ] 1. Fully granulating
- [ ] 2. Early/partial granulation
- [ ] 3. Not healing

Skin Color
- [x] Within Normal Limits
- [ ] Pale
- [ ] Jaundice
- [ ] Cyanotic

Skin Turgor:
- [x] Good
- [ ] Fair
- [ ] Poor

Skin Abnormalities:
- [x] Dry
- [x] Warm
- [ ] Cool
- [ ] Rash
- [ ] Ulcer
- [ ] Ostomy
- [x] Incision
- [ ] Diaphoretic
- [ ] Others (Specify):

Additional Skin Comments

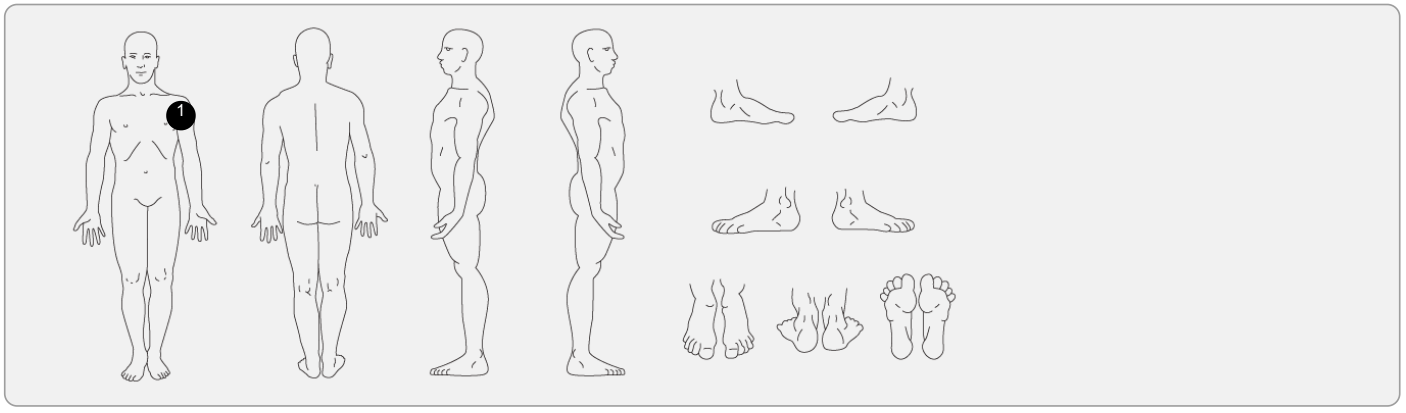
Norton Scale for Pressure Ulcer Risk Assessment

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	○ Good	● Fair	○ Poor	○ Very Bad
Mental Condition	● Alert	○ Apathetic	○ Confused	○ Stuporous
Activity	● Ambulant	○ Walks With Help	○ Chairbound	○ Bedfast
Mobility	○ Full	● Slightly Impaired	○ Very Limited	○ Immobile
Incontinence	● None	○ Occassional	○ Usually Urinary	○ Urinary and Fecal
			Total Score:	18

Wound

Wound Worksheet

Fetch Wounds From Previous Wound Note



Wound 1

Location	Size	Pressure Injury Stage
left shoulder	0 x 0 x 0	-
Type	**Drainage**	**Amount Of Drainage**
Surgical	None	None
Odor	**Color**	**Tunneling/Undermining**
None	-	-
Tunneling Location	**Tunneling Length (cm)**	**Undermining Location**

-	-	-
Undermining Length (cm)	Surrounding Tissue	Wound Base
-	Normal	-
Level Of Pain	Edema	Stoma
4	-	-
Slough and/or Echar	Slough (%)	Eschar (%)
-	-	-
Granulation (%)	Pink/Red (%)	Necrotic (%)
-	-	-
Wound Status	Wound Description	Satisfaction of Return Demonstration
Non-Healing	-	Yes - Needs Additional Education

Patient's Response to Wound Care

Fair - Patient tolerated the assessment process of this SOC visit.

Wound Care

Using an aseptic technique: Big bandage can come off 3/27/2026 or 3/28/2026. Leave big, yellow mesh on. Cover with 4x4 to keep protected. Keep the dressing clean, dry and intact, until MD appointment. Can shower starting 3/28/2026, just rinse/let water run and pat dry. Keep left arm in sling, except when showering.

Onset Date

2026-03-24

SAFETY & RISK ASSESSMENT

Home Safety Assessment

Home Environment

- [ ] Architectural Barriers
- [ ] No fire extinguisher
- [ ] No emergency exit plan
- [ ] Medications are improperly stored
- [ ] Inadequate electricity / cooling / heating
- [ ] Rodent / insect infestation
- [ ] No telephone
- [ ] Inadequate supply of water
- [ ] Unsafe electrical wiring
- [ ] Cluttered surroundings
- [ ] Inadequate sanitation / plumbing
- [ ] Nonworking stove / refrigerator
- [ ] Inadequate railing
- [ ] Inadequate smoke detectors on all levels of home
- [ ] Exposed flammables
- [ ] Unsafe oxygen use
- [ ] Unsafe storage of equipments
- [ ] Inadequate Lighting

Other:

Type of Assistance Patient Receives - other than from Home Health Agency Staff

ADL(Bathing, dressing, toileting, eating/feeding, bowel)	[x] Family/Friends	[ ] Provider Services	[ ] Paid Caregiver	[ ] Volunteer Organizations
IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances)	[x] Family/Friends	[ ] Provider Services	[ ] Paid Caregiver	[ ] Volunteer Organizations
Psychosocial Support	[x] Family/Friends	[ ] Provider Services	[ ] Paid Caregiver	[ ] Volunteer Organizations
Assistance with doctors appointment, delivery of medications	[x] Family/Friends	[ ] Provider Services	[ ] Paid Caregiver	[ ] Volunteer Organizations
Management of Finances	[x] Family/Friends	[ ] Provider Services	[ ] Paid Caregiver	[ ] Volunteer Organizations

Comments:

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non-agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety (for example, due to cognitive impairment)

- ( ) **0. No assistance needed – patient is independent or does not have needs in this area**
- ( ) **1. Non-agency caregiver(s) currently provide assistance**
- (x) **2. Non-agency caregiver(s) need training/supportive services to provide assistance**
- ( ) **3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance**
- ( ) **4. Assistance needed, but no non-agency caregiver(s) available**

Safety Hazards Affecting Patient

- [ ] Stairs
- [ ] No running water, plumbing
- [ ] Inadequate lighting, heating and cooling
- [ ] Narrow or obstructed walkway
- [ ] Insect/Rodent Infestation
- [ ] Lack of fire safety devices
- [ ] No gas/electric appliance
- [ ] Cluttered/soiled living area
- [x] No hazards identified

Others:

Safety Measures

- ( ) 1 - Bleeding precautions
- ( ) 2 - O_2 precautions
- ( ) 3 - Seizure precautions
- (x) 4 - Fall precautions
- ( ) 5 - Aspiration precautions
- ( ) 6 - Siderails up
- ( ) 7 - Elevate head of bed
- ( ) 8 - 24 hr. supervision
- (x) 9- Clear Pathways up
- ( ) 10 - Lock w/c with transfers
- (x) 11 - Infection control measures
- ( ) 12 - Walker/cane
- ( ) 13 - Sharps Precaution
- (x) 14 - Other:

Covid 19 precautions
Universal precaution/911 protocol

Hospitalization Risk Assessment

Prior Pattern: Check all that apply

- [x] > 1 Hospitalizations or ER visits in the past 12 months
- [x] History of falls

Chronic conditions: Check all that apply (M1020/1022/1024)

- [ ] HF
- [ ] Diabetes
- [ ] COPD
- [ ] Chronic skin ulcers (Wound consult if indicated for any wounds)
- [ ] HIV/AIDS

Risk factors: (Check all that apply)

- [x] Discharged from hospital or skilled nursing facility
- [x] More than 2 secondary diagnoses
- [ ] Low socioeconomic status or financial concerns ♦
- [ ] Lives alone ►♦
- [ ] Inadequate support network ♦
- [x] ADL assistance needed ►
- [x] Home safety risks ► ♦
- [ ] Dyspnea ► ★
- [x] Help with managing medications needed ► ★
- [ ] Non-compliance with medication regimen ♦ ★
- [ ] Confusion ♦ ★
- [ ] Pressure ulcer ★
- [ ] Stasis ulcer ★
- [ ] Overall Poor Status/Prognosis ■
- [ ] Low literacy level ♦
- [ ] Depression ♦

- [x] ►Consider Therapy referral (PT, OT, ST)
- [x] ♦Consider MSW referral
- [ ] ■ Consider Hospice referral
- [ ] ★Consider RN referral, if not ordered

Total number of checked boxes is:
8

Your agency may want to select a threshold score to target patients at high risk.

(For example: 5 or greater risk factors may indicate that the patient is at risk for hospitalization. Note: This number is for convenience only and has not been tested or validated. The agency may modify the score based upon the needs of their patient population.)

Carry out patient specific interventions as appropriate/ordered, if patient is at risk for hospitalization: (Coordinate with M2250)

Referrals:

- [x] SN
- [x] PT
- [ ] OT
- [ ] ST
- [ ] MSW
- [ ] HHA
- [ ] Dietary Consultant

Other:

Consider implementing any of the following interventions, if patient is at risk for hospitalization:

- [x] Medication Management
- [x] Medication Reconciliation
- [x] Assess patient's: knowledge, ability, resources and adherence
- [x] Education
- [x] Patient/family education
- [ ] Enrollment into a disease management program

(Specify):

- [ ] Hospice/Palliative Referral
- [ ] Phone Monitoring
- [ ] Front-loading Visits
- [x] Individualized Patient Emergency Care Plan
- [ ] Telemonitoring
- [x] Care Coordination (physicians, hospitals, nursing homes ...)
- [x] Fall Prevention Program
- [ ] Wound Care

Immunizations (M1040, M1045, M1050, M1055)

- [ ] Influenza
- [ ] Pneumococcal

Fall Prevention Program:
>Discussed fall preventative measures: slow position changes, ask for help as needed, keep adequate lighting, keep the pathway clear and clutter-free, and use an assistive device
>Referred to Physical therapy for eval/treatment.

Other:

(M1033) Risk for Hospitalization: Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- [ ] 1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- [ ] 2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- [ ] 3. Multiple hospitalizations (2 or more) in the past 6 months
- [ ] 4. Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5. Decline in mental, emotional, or behavioral status in the past 3 months
- [x] 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- [x] 7. Currently taking 5 or more medications
- [x] 8. Currently reports exhaustion
- [ ] 9. Other risk(s) not listed in 1 - 8
- [ ] 10. None of the above

Additional Risk Information:

Prior Hospitalization

- (●) Yes
- ( ) No

Number of times: 1

FALL RISK ASSESSMENT TOOL

MAHC 10* - FALL RISK ASSESSMENT TOOL

	Yes	No
Age 65+	✓	
Diagnosis (3 or more co-existing) Includes only documented medical diagnosis	✓	
Prior history of falls within 3 months An unintentional change in position resulting in coming to rest on the ground or at a lower level		✓
Incontinence Inability to make it to the bathroom or commode in timely manner. Includes frequency, urgency, and/or nocturia.		✓
Visual impairment Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tolerance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.		✓

Impaired functional mobility May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, impaired coordination or improper use of assistive devices.	○	✓
Environmental hazards May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.	○	✓
Poly Pharmacy (4 or more prescriptions - any type) All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquilizers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.	✓	○
Pain affecting level of function Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.	✓	○
Cognitive impairment Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsivity, memory deficits. Consider patients ability to adhere to the plan of care.	○	✓
A score of 4 or more is considered at risk for falling	4	

Fire Assessment for Patient with Oxygen

☑ **Patient is not using oxygen**

Does patient have No Smoking signs posted?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Does patient or anyone in the home smoke with oxygen in use?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Are smoke detectors present and working properly?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Does patient have a properly functioning fire extinguisher?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Are oxygen cylinders stored properly?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Are all electrical cords near oxygen intact and free for fraying?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Does patient have an evacuation plan in case of fire?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Are all cleaning fluids and aerosols stored away from oxygen, and not used when oxygen is in use?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Does patient refrain from using petroleum products around oxygen?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**
Does patient only used water-based body and lip moisturizers?	○ **Yes**	○ **No**
• **Are the Patient and Caregiver educated regarding this?**	○ **Yes**	○ **No**

Comments:

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities: Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

- **3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper
- **2. Needed Some Help** – Patient needed partial assistance from another person to complete activities
- **1. Dependent** – A helper completed the activities for the patient.
- **8. Unknown**
- **9. Not Applicable**

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	✓	○	○	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	✓	○	○	○	○
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	✓	○	○	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	✓	○	○	○	○

(GG0110) Prior Device Use

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (**Check all that apply)**

- ☐ **A. Manual wheelchair**
- ☐ **B. Motorized wheelchair and/or scooter**
- ☐ **C. Mechanical lift**
- ☐ **D. Walker**
- ☐ **E. Orthotics/Prosthetics**
- ☑ **Z. None of the above**

(GG0130) Self Care

(GG0130) SOC/ROC Performance

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.
If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

	A. Eating	B. Oral Hygiene	C. Toileting Hygiene	E. Shower /bathe self	F. Upper body dressing	G. Lower body dressing	H. Putting on /taking off footwear

(06) Independent	○	○	○	○	○	○	○
(05) Setup or clean-up assistance	✓	✓	○	○	○	○	○
(04) Supervision or touching assistance	○	○	✓	✓	✓	✓	○
(03) Partial/moderate assistance	○	○	○	○	○	○	✓
(02) Substantial/maximal assistance	○	○	○	○	○	○	○
(01) Dependent	○	○	○	○	○	○	○
(07) Patient refused	○	○	○	○	○	○	○
(09) Not applicable	○	○	○	○	○	○	○
(10) Not attempted due to environmental limitations	○	○	○	○	○	○	○
(88) Not attempted due to medical conditions or safety concerns	○	○	○	○	○	○	○
(-) Not assessed/no information	○	○	○	○	○	○	○

(M1800) Grooming

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- ○ **0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.**
- ● **1. Grooming utensils must be placed within reach before able to complete grooming activities.**
- ○ **2. Someone must assist the patient to groom self.**
- ○ **3. Patient depends entirely upon someone else for grooming needs**

(M1810) Ability to Dress Upper Body

Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:

- ○ **0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.**
- ○ **1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.**
- ● **2. Someone must help the patient put on upper body clothing.**
- ○ **3. Patient depends entirely upon another person to dress the upper body.**

(M1820) Ability to Dress Lower Body

Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:

- ○ **0. Able to obtain, put on, and remove clothing and shoes without assistance.**
- ○ **1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.**
- ● **2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.**
- ○ **3. Patient depends entirely upon another person to dress lower body.**

(M1830) Bathing

Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).**

- ○ **0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.**
- ● **1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.**
- ○ **2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.**
- ○ **3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.**
- ○ **4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.**
- ○ **5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person**
- ○ **6. Unable to participate effectively in bathing and is bathed totally by another person.**

(M1840) Toilet Transferring

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ○ **0. Able to get to and from the toilet and transfer independently with or without a device**
- ● **1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.**
- ○ **2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).**
- ○ **3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently**
- ○ **4. Is totally dependent in toileting.**

(M1845) Toileting Hygiene

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ○ **0. Able to manage toileting hygiene and clothing management without assistance.**
- ● **1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.**
- ○ **2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.**
- ○ **3. Patient depends entirely upon another person to maintain toileting hygiene**

(M1850) Transferring

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ○ **0. Able to independently transfer.**
- ● **1. Able to transfer with minimal human assistance or with use of an assistive device.**
- ○ **2. Able to bear weight and pivot during the transfer process but unable to transfer self.**
- ○ **3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person**
- ○ **4. Bedfast, unable to transfer but is able to turn and position self in bed.**
- ○ **5. Bedfast, unable to transfer and is unable to turn and position self**

(M1860) Ambulation/Locomotion

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ○ **0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).**
- ○ **1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.**
- ○ **2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.**
- ● **3. Able to walk only with the supervision or assistance of another person at all times.**
- ○ **4. Chairfast, unable to ambulate but is able to wheel self independently.**
- ○ **5. Chairfast, unable to ambulate and is unable to wheel self**
- ○ **6. Bedfast, unable to ambulate or be up in a chair.**

FUNCTIONAL ASSESSMENT (MOBILITY & MUSCULOSKELETAL)

(GG0170) Mobility

(GG0170A) A. Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170B) B. Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170C) C. Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170D) D. Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170E) E. Chair/bed-to-chair transfer: The ability to transfer to and from a bed to a chair (or wheelchair).

SOC/ROC Performance

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170F) F. Toilet transfer: The ability to get on and off a toilet or commode.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170G) G. Car transfer: The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170I) I. Walk 10 feet: Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)*

SOC/ROC Performance

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns: Once standing, the ability to walk 50 feet and make two turns.

SOC/ROC Performance

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170K) K. Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ● (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

SOC/ROC Performance

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170M) M. 1 step (curb): The ability to go up and down a curb and/or up and down one step. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170N) N. 4 steps: The ability to go up and down four steps with or without a rail. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170O) O. 12 steps: The ability to go up and down 12 steps with or without a rail.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ● (-) Not assessed/no information

(GG0170P) P. Picking up object: The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

- ● 0. No **[GOTO M1600]**
- ○ 1. Yes
- ○ Not assessed (no information)

(GG0170R) R. Wheel 50 feet with two turns: Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.

SOC/ROC Performance

- ○ 1. Manual
- ○ 2. Motorized
- ○ Not assessed (no information)

(GG0170S) S. Wheel 150 feet: Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.

SOC/ROC Performance

- ○ 1. Manual
- ○ 2. Motorized
- ○ Not assessed (no information)

Gait and Mobility Abnormalities

- ☐ No Problems Identified
- ☑ Weakness
- ☐ Bed Bound
- ☐ Joint Stiffness
- ☑ Orthopedic Aftercare
- ☐ Weight Bearing Restriction
- ☐ Fracture:
- ☐ Amputation:
- ☐ Paresthesia:
- ☐ Foot Drop: ☐ Right ☐ Left
- ☑ Poor Balance
- ☐ Chair Bound
- ☐ Shuffling/Wide-based Gait
- ☑ High Risk for Falls
- ☐ Poor Conditioning
- ☐ Joints Pain:
- ☐ Atrophy:
- ☐ Contractures: Site

Fall(s) since last visit (Specify):
0

Weakness (Specify):

Change in ADL (Explain):

Comments:

Range of Motion

- ☑ Limited Range of Motion: ☐ RUE ☐ RLE ☑ LUE ☐ LLE

☐ Quadriplegia ☐ Hemiplegia ☐ Paraplegia

Comments:
Patient is status post left shoulder repair. Surgical site appears consistent with expected post-operative presentation (adjust if you need to note dressing, swelling, or restrictions). Patient reports manageable discomfort and is following post-op precautions as instructed. No signs of infection or complications observed or reported.

Grip Strength

☑ Normal ☐ Strong ☐ Weak ☐ Equal
☑ Unequal ☐ Bilateral ☐ Right ☑ Left

Others (Specify):

Grasp Strength

☑ Normal ☐ Strong ☐ Weak ☐ Equal ☑ Unequal

Comments:

DME and Supplies

DME Lookup

Arm Sling for immobilization, but can be removed on showering

#18B ACTIVITIES PERMITTED

- ○ 1- Complete bedrest
- ○ 2 - Bedrest/BRP
- ● 3 - Up as tolerated
- ○ 4 - Transfer bed/chair
- ● 5 - Exercises prescribed
- ○ 6 - Partial weight bearing
- ○ 7 - Independent in home
- ○ 8 - Crutches
- ○ 9 - Cane
- ○ A - Wheelchair
- ○ B - Walker
- ○ C - No restrictions
- ● D - Other (specify)

Arm Sling

ENDOCRINE (DIABETIC MANAGEMENT)

Endocrine

Is patient diabetic?	○ Yes	● No	
Insulin dependent?	○ Yes	○ No	How Long?
Is patient independently able to draw up correct dose of insulin?	○ Yes	○ No	○ N/A
Is patient able to administer own insulin?	○ Yes	○ No	○ N/A
Is patient taking oral hypoglycemic agent?	○ Yes	○ No	○ N/A
Is patient independent with glucometer use?	○ Yes	○ No	○ N/A
Is caregiver able to draw up and administer insulin?	○ Yes	○ No	○ N/A
Is caregiver independent with glucometer use?	○ Yes	○ No	○ N/A
Does patient or caregiver routinely perform inspection of the patient's lower extremities?	○ Yes	○ No	○ N/A

Diabetic Management

☐ Diet ☐ Hypoglycemic ☐ Exercise ☐ Insulin

Insulin Administered By:

☐ N/A ☐ Caregivier ☐ Patient ☐ Skilled Nurse

S&S of Hyperglycemia

☐ Polyuria ☐ Polyphagia ☐ Polydipsia ☐ Blurred Vision ☐ Fatigue

Others (Specify):

S&S of Hypoglycemia

☐ Perspiration ☐ Dizziness ☐ Irritability ☐ Pale ☐ Shakiness

Others (Specify):

Blood glucose range: **Low to High (mg/dL):**

Comments:

PLAN OF CARE AND SKILLED INTERVENTIONS

Plan of Care Orders

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 1x/wk x 4wks	**MSW Frequency:**	**PT Frequency: For evaluation and treatment effective on week of 4/5/2026.**	**OT Frequency:**
HHA Frequency:	**ST Frequency**	**Dietitian Frequency:**	

MD SIGNATURE COVERS ALL PAGES

ADMIT PATIENT TO STAR HOME HEALTH CARE INC.
ALL DIAGNOSES AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

The patient is homebound due to his medical status post-surgery that include significant functional limitations, particularly in mobility and self-care activities due to pain, swelling, and the need for rehabilitation of the affected shoulder. His ability to perform daily activities will be compromised, necessitating support in the home environment. This aligns with Medicare's home health admission guidelines, which stipulate that patients must demonstrate a need for skilled care and have difficulty performing self-care tasks. The skilled care required includes physical therapy, which is essential for regaining strength and range of motion in the shoulder, and skilled nursing to manage post-operative care, including wound care and monitoring for any complications. The medical necessity of these services is underscored by the need for ongoing assessment and intervention to ensure proper healing and restoration of function following surgery. Furthermore, the patient's recent hospital discharge and the nature of his surgery highlight the need for a structured home health care plan. His recovery will benefit from the professional oversight of skilled nurses and therapists to mitigate risks, optimize rehabilitation efforts, and support his transition back to independence. Therefore, admitting this patient to home health care will provide the comprehensive support he requires to achieve optimal recovery outcomes in the comfort of his home.

FREQUENCY AND DURATION:
HH SN: 1 time per week for 4 weeks for skilled observation and assessment, wound status assessment, medication review and reconciliation, pain management, infection control measures, diet, home safety, fall prevention, and disease process education.
HH PT: Evaluation and treatment for mobility, gait, transfer, establish therapeutic exercise, strengthening, and fall prevention techniques training effective on week of 4/5/2026. PT order: Movement of the hand and wrist and a little bit of the elbow for now. The rest don't do anything else unless doctor told patient differently.

WOUND CARE ORDER:
Surgical wound at Left Shoulder: Using aseptic technique, as the big bandage can come off by 3/27/2026 or 3/28/2026. Leave the big, yellow mesh on the incision. Cover with gauze 4x4 dressing to keep protect. SN to instruct the patient can shower starting 3/28/2026, just rinse/let water run and pat dry. Keep left arm in sling, except when showering.

Teachings/Interventions

Skilled Intervention

Assessment/Instruction/Performance:
Patient is alert and oriented ×4. Reports manageable pain and remains medication compliant. Denies new concerns at this time. Patient and wife were receptive to education and asked appropriate questions.

Objective:
Dressing removed today; yellow mesh left in place per post-operative protocol.
Surgical incision assessed and appeared to be healing appropriately. No redness noted, incision not warm to the touch, and no drainage observed.
Pain level reported as 4/10 during assessment.
Patient is status post left shoulder repair and continues to follow post-operative precautions.
Patient is eating, though slightly less than baseline due to recent surgery. Hydration adequate.
Patient is currently on opioids for pain management and is taking bowel medications as ordered to prevent constipation.

Education Provided:
Wife educated on showering instructions: patient may shower, allow water to run over the incision, avoid soap directly on the site, and pat dry afterward.
Wife was shown and instructed on how to reapply a new dressing after showering.
Reviewed signs and symptoms of infection, including redness, warmth, swelling, drainage, fever, or increased pain.
Fall-prevention education was provided to both the patient and the wife, including safe mobility practices and call-light use.
Discussed bowel regimen due to opioid use and reinforced the importance of hydration and monitoring bowel movements.

Assessment:
Patient is stable, cooperative, and engaged in care. Incision healing appropriately with no signs of infection. Pain is present but manageable. Patient and wife demonstrated good understanding of all education provided and verbalized readiness to perform dressing changes as instructed. No safety concerns noted at this time.

Intervention:
Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment of all body systems with OASIS elements evaluation was initiated. Reconciled facility discharges medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed the plan of care draft with the patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial discussion on the management of other disease processes will be done on the next visit.

SN assessed patient status post left total shoulder replacement, noting alert and oriented ×4, stable overall condition, and report of left shoulder pain rated 4/10 described as throbbing and increased with movement. Surgical site evaluated with dressing intact per orders, no redness, drainage, or abnormal swelling observed; yellow mesh dressing in place as instructed. Patient using sling appropriately with limited movement to hand, wrist, and slight elbow motion only. SN provided skilled instruction on post-operative precautions including strict adherence to activity limitations, avoidance of lifting/pushing/pulling, and maintaining proper sling positioning except during hygiene. Education provided on wound care technique, including covering with clean 4x4 dressing, allowing water to run over site during showering, and patting dry without friction. Pain management strategies discussed including scheduled rest periods, proper repositioning using pillows for support, and use of deep breathing exercises and diversion techniques such as listening to music or light activity to reduce discomfort perception. Medication regimen reviewed with emphasis on correct opioid use, timing, and safety precautions including fall risk, constipation prevention, and avoiding unsupervised dose changes. Patient instructed on importance of hydration, small frequent meals due to decreased appetite, and maintaining adequate protein intake to support healing. Caregiver instructed on assisting with ADLs, ensuring safe environment, and observing for signs of complications such as increased pain, swelling, or drainage to report promptly.

SN assessed functional mobility and safety awareness in the home, noting patient requires assistance of another person to leave residence and has history of falls, placing patient at increased risk for injury. No assistive device currently in use; gait observed as cautious with need for supervision due to post-surgical limitation and balance concerns. SN provided instruction on fall prevention strategies including use of non-skid footwear or socks with traction, removal of clutter and loose rugs, ensuring clear pathways, and adequate lighting throughout the home. Education provided on maintaining proper body mechanics and avoiding sudden movements that could compromise joint integrity. Patient and caregiver instructed on safe transfer techniques and gradual position changes to prevent instability. Reinforced importance of adhering to prescribed physical therapy exercises limited to hand, wrist, and slight elbow movement only unless otherwise directed by physician. SN also reviewed breathing techniques including deep and pursed-lip breathing to promote relaxation and prevent exertion during activity, despite patient denying shortness of breath. Education provided on energy conservation techniques such as pacing activities, incorporating rest breaks, and avoiding overexertion. Caregiver instructed to supervise mobility and assist as needed to prevent falls and protect surgical joint. Patient verbalized understanding and demonstrated willingness to comply with safety measures and rehabilitation plan.

SN assessed patient's pain status, noting report of left shoulder pain rated 4/10, described as throbbing and occurring more frequently with activity and movement. Patient observed guarding left upper extremity and maintaining sling as instructed. SN provided skilled instruction on pharmacologic and non-pharmacologic pain control measures, including proper timing and use of prescribed opioid and aspirin, emphasizing adherence to dosing schedule, avoiding self-adjustment, and awareness of side effects such as drowsiness and constipation. Education provided on opioid safety including avoiding unassisted ambulation when feeling sedated and securing medication to prevent misuse. Non-pharmacologic strategies implemented including positioning with pillows to support the affected arm, scheduled rest periods between activities, and use of relaxation techniques such as deep breathing exercises and distraction methods (e.g., music, television). SN instructed patient to perform gentle hand, wrist, and limited elbow movements only as ordered to prevent stiffness without increasing pain. Education also provided on recognizing worsening pain or unrelieved discomfort and reporting changes promptly. Caregiver instructed to assist with positioning, encourage adherence to pain regimen, and observe for signs of over-sedation or ineffective pain control.

SN assessed home environment and patient's mobility status, identifying increased fall risk related to recent surgery, history of falling, use of opioid medication, and need for assistance when leaving the home. Patient ambulating cautiously without assistive device but requiring supervision for safety. SN provided skilled instruction on fall prevention measures including wearing non-skid socks with traction, maintaining clean and uncluttered walkways, securing or removing loose rugs, and ensuring adequate lighting especially at night. Education provided on slow position changes from sitting to standing to prevent loss of balance, and importance of asking for assistance during ambulation and transfers. SN instructed patient to avoid carrying items while walking to maintain balance and protect surgical site. Caregiver instructed on maintaining a safe home setup, providing standby assistance during mobility, and observing for signs of unsteadiness or medication-related dizziness. Energy conservation techniques discussed, encouraging pacing of activities with rest periods to reduce fatigue-related falls. Patient and caregiver verbalized understanding and agreed to implement safety recommendations to reduce fall risk.

☑ **Response to Teaching/Interventions**

Verbalized Understanding	☑ **Patient (%):** 80% ☑ **Caregiver (%):** 90%
Return Demonstration	☐ **Patient (%):** ☐ **Caregiver (%):**
Require Further Teaching	☐ **Patient** ☐ **Caregiver**

Comments:
The patient verbalized 80% of understanding of the disease process taught during this visit. Patient able to teach back 3 out of 4 non-pharmacologic pain management measures and fall prevention techniques. CG verbalized 90% understanding and is available to assist the patient for safety with ADLs/IADLs, medication, and diet compliance for the patient. The patient and caregiver are calm, relaxed, and comfortable at the end of the visit.

Rechecked pain level post visit reduces to 2/10.

Instructions during visit

- [x] Assess/Observe
- [ ] Injection Administration
- [ ] Catheter change
- [ ] IV Therapy
- [ ] Wound Care
- [x] Teaching/Instructions

Patient Strength

- [x] Motivated Learner
- [x] Strong Support System
- [ ] Absence of Multiple Diagnosis
- [ ] Enhanced Socioeconomic Status

Other:

Conclusions

- [x] Skilled Intervention Needed
- [x] Skilled Instruction Needed
- [ ] No Skilled Service Needed

Other:

Rehabilitation Potential

- [ ] Rehabilitation potential is poor
- [ ] Rehabilitation potential is good
- [x] Rehabilitation potential is fair
- [ ] Rehabilitation potential is excellent

Others:

Conference With

- [x] MD
- [ ] SN
- [x] PT
- [ ] OT
- [ ] ST
- [ ] MSW
- [ ] HHA
- [x] DON/DOPCS
- [ ] Clinical Coordinator
- [x] Clinical Manager

Name:
Aimee Cara Floresca RN, CM

Regarding:
Start of care, Plan of care, visit frequencies, and update of patient health condition.

Physician Contacted Re:
Patient's admission to Star HH, updated health condition, and visit frequencies.

Order Changes:
None

Plan for Next Visit:
Will continue to assess and evaluate the wound status, level of pain, patient understanding of the disease process, functional status, knowledge of self-care management, and overall patient health condition and reinforce teachings on the next visit.

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge: §484.60 (2.xiii)
Initiated patient and caregiver education and training to facilitate timely discharge.

Discharge Plans

Discharge when patient is independent in management of medical needs.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge to care of physician.
Discharge when reliable caregiver available to assist with patient's medical needs.
When patient knows when to notify physician.
Able to stay in residence with primary caregiver's assistance or community agency's support.
When goals are met.
Able to return to an independent level of care.
Discharge when goals met. Discharge when wound(s) healed.

- [ ] Written notice of discharge provided to patient

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:

- [x] Patient/Family
- [ ] Care Manager
- [ ] Physician
- [ ] Other (specify):

Discharge Plan:

- [x] To self-care when goals met
- [x] To self care when max potential achieved
- [x] To outpatient therapy with MD approval
- [ ] Others (Specify):

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

A 69-year-old male was admitted to home health services following discharge from an acute care hospital after a total left shoulder replacement. He is under the care of a physician and was evaluated by a skilled nurse to initiate services. The patient is considered homebound due to post-surgical weakness, pain, and limited endurance, requiring significant effort to leave the home and necessitating extended periods of rest. Primary diagnosis is aftercare following joint replacement, with additional diagnoses including presence of left artificial shoulder joint, hypertension, mixed hyperlipidemia, benign prostatic hyperplasia, and history of transient ischemic attack. The patient resides with others who provide support, though caregiver training is needed to assist with care needs.

On assessment, vital signs were within normal limits with temperature of 98.1°F, pulse 73 beats per minute, blood pressure 129/78 mmHg, and respiratory rate 18 breaths per minute. Cardiopulmonary status is stable. The patient reports decreased appetite related to recent surgery but is tolerating oral intake. Surgical site is covered with a non-removable dressing, with no visible concerns reported. Skin integrity remains intact with no evidence of pressure injuries. Cognitive assessment indicates the patient is alert and oriented with a BIMS score of 12, demonstrating adequate understanding and ability to participate in care and education. Psychosocial status is stable, with no reported concerns related to mood or isolation.

Functionally, the patient demonstrates moderate limitations, requiring assistance with grooming, dressing, and toileting hygiene, while able to bathe independently using assistive devices. Transfers require minimal assistance, and ambulation is limited to short distances indoors with supervision. The patient is prescribed multiple medications, including high-risk agents such as opioids and antiplatelet therapy, and has a history of difficulty with medication adherence, though able to follow regimen when medications are pre-arranged by a caregiver. Skilled nursing services are indicated to support post-surgical recovery, provide education on medication safety, reinforce safe mobility techniques, and train the caregiver to assist with activities of daily living. A visit frequency of two to three times per week is appropriate to address ongoing care needs and promote recovery.

Patient Care Plan / Goals / Interventions:

Problem: Vital Signs Assessment
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Unmet
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: SN
Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet
Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness.
Progress Toward Goals:
No Progress Yet

Problem: Aftercare following joint replacement surgery
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Achieve full range of motion in the joint
Interventions: Assigned Staff: SN
Assess for signs of infection at the surgical site.
Progress Toward Goals:
No Progress Yet

Problem: Presence of left artificial shoulder joint
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Maintain joint integrity and function.
Interventions: Assigned Staff: SN
Educate the patient on proper shoulder care and precautions.
Progress Toward Goals:
No Progress Yet
Assess and document any changes in shoulder mobility or discomfort.
Progress Toward Goals:
No Progress Yet

Problem: Essential (primary) hypertension
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Maintain blood pressure within normal ranges.
Interventions: Assigned Staff: SN
Implement regular blood pressure checking and record keeping.
Progress Toward Goals:
No Progress Yet
Coordinate with the physician for medication management and adjustments.
Progress Toward Goals:
No Progress Yet

Problem: Mixed hyperlipidemia
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Achieve and maintain optimal lipid levels.
Interventions: Assigned Staff: SN
Develop a nutrition plan focused on heart-healthy foods.
Progress Toward Goals:
No Progress Yet
Educate the patient on the importance of medication adherence for lipid management.
Progress Toward Goals:
No Progress Yet

Problem: Benign prostatic hyperplasia without lower urinary tract symptoms
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Maintain urinary function and comfort.
Interventions: Assigned Staff: SN
Educate the patient on bladder health and fluid management.
Progress Toward Goals:
No Progress Yet
Check for any changes in urinary habits or discomfort.
Progress Toward Goals:
No Progress Yet

Problem: Personal history of TIA and cerebral infarction without residual deficits
Goals: Target Completion: 4 7 Term: Long-term Status: Unmet
Prevent future cerebrovascular incidents.
Interventions: Assigned Staff: SN
Educate the patient and family on risk factors and lifestyle modifications.
Progress Toward Goals:
No Progress Yet
Encourage adherence to prescribed medications for stroke prevention.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Unmet
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (03/29/2026):

Assigned Staff: Lara, Toni RN

Electronically Signed by Lara, Toni RN 03/29/2026